



Armada Health Service

Code Blue – Medical Emergency

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1	New Document	1/9/2022	CNS Resuscitation	Emergency Management Committee Governance and Risk Committee
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Introduction

A code blue medical emergency is activated when there is an acute deterioration in the condition of a patient, visitor or staff member. A Medical Emergency Team (MET) will respond to all cardiac and/or respiratory arrests, and to the sudden collapse or deterioration of a patient, visitor or staff member within Armadale Health Service (AHS) and the immediate grounds within the perimeter of the hospital site.

The MET is a designated group of healthcare clinicians, who can be assembled quickly to deliver critical care expertise in response to clinical deterioration of a person and initiate appropriate clinical management. This forms a part of the rapid response system for the Armadale Health Service.

Systems are in place at AHS to improve the recognition and response to clinical deterioration (RRCD) of inpatients, to ensure the delivery of appropriate and timely care. A medical emergency, such as cardiac and/or respiratory arrest may not be an isolated event, but an end point in a process of clinical deterioration and physiological compromise.

Boundaries and response

- Include a list of buildings where the MET may respond.

The Primary MET areas (which are indicated by Green Shade on the MAP)

- The primary MET area includes all the Clinical areas, inclusive of outpatient units within the boundary, from the Leschen Unit, including the Carl Streich unit, to the Galliers wing.
- The Secondary MET response areas are shaded orange. The MET Team will respond to these areas; however, St John's Ambulance (SJA) should be considered in transporting those patients to the Emergency Department. This involves all car park areas (inclusive of staff parking), Mead Centre and J-Block community centre
- Any areas outside the above mentioned will be considered as out of bounds and will need mandatory SJA involvement and MET team will not be attending them. Hence, a member of the public or staff will need to call "000" emergency phone service to acquire assistance.

RESUSCITATION TROLLEY LOCATIONS

LOCATION	TYPE OF DEFIBRILLATOR
Emergency department Resus Bay 2	Manual/AED with pacing capabilities
Emergency department Resus Bay 1	Manual/AED with pacing capabilities
Emergency department Bay 10-15	Manual /AED
Emergency department General assessment area	Manual /AED
Radiology	Manual/AED
Rehabilitation ward	Manual /AED
Community Rehabilitation Unit	Manual /AED
Paediatric outpatient clinic	Manual /AED
Antenatal Clinic (J Block)	Manual /AED
Medical Admission Unit Bed 1-16	Manual /AED
Medical Admission Unit 17-32	Manual /AED
Operating Theatre main complex	Manual /AED
Operating Theatre 7	Manual /AED
Operating theatre 6	Manual /AED with pacing capabilities
Canning Medical ward	Manual /AED
Colyer Medical-Surgical ward	Manual /AED
ICU	Manual/AED with pacing capabilities
Paediatric (Campbell) ward	Manual/AED
Dialysis Unit	Manual/AED
Same day Unit	Manual/AED
Banksia ward	Manual/AED
Moodjar ward	Manual/AED

MET TEAM PRIMARY STRUCTURE

ROLE	ADULT		PAEDIATRIC		NEONATAL	
	IN HOURS	OUT OF HOURS	IN HOURS	OUT OF HOURS	IN HOURS	OUT OF HOURS
TEAM LEADER	ICU Registrar	ICU Registrar	Consultant Paediatrician	Paediatric Registrar	Consultant Paediatrician	Paediatric Registrar
AIRWAY LEAD	Anaesthetic Registrar	Anaesthetic Registrar	Anaesthetic Registrar	Anaesthetic Registrar	Anaesthetic Registrar	Anaesthetic Registrar
MEDICAL LIASION	General Medicine Registrar	General Medicine Registrar	Paediatric Registrar	ICU Registrar	Paediatric Registrar	ICU Registrar
NURSING LEAD	CNS-Resuscitation	CNS-After Hours	NUM Paediatrics and Neonates	CNS-After Hours	NUM Paediatric and Neonatal	CNS-After Hours
NURSING TEAM	WARD CNS, Primary Nurse and Shift coordinator	Shift coordinator, Primary Nurse, Senior ward Nurse	Shift coordinator or Primary Nurse Senior Nurse	Shift coordinator, Primary Nurse, Senior Nurse	Shift coordinator, Primary Nurse, Senior Nurse	Shift coordinator, Primary Nurse, Senior Nurse
LOGISTICS	PCA	PCA	PCA	PCA	PCA	PCA

If the MET call occurs in a secondary MET area, a resuscitation competent Nurse from the Emergency department will be attending the code Blue with an ED PCA who brings a MET bag.

MET-HANDOVER

The overnight after-hours clinical nurse specialist (AHCNS) conducts an ISOBAR handover to the morning MET Clinical Nurse Specialist (MET-CNS) regarding patients with MET calls overnight. The handover process also occurs at designated times during the afternoon and night shifts. The handover times are as follows:

- Morning Handover: 0700 Hrs.
- Midday Handover: 1300 Hrs.
- Evening Handover: 2100 Hrs.

Activation criteria

The following section outlines the activation criteria for a Code Blue – Medical Emergency for the various patient cohorts.

Adult – Code Blue

Call an Adult Code Blue Medical Emergency if the following criteria are identified:

- Threatened airway
- Bradypnea (<5 bpm) or tachypnoea (>36 bpm)
- Hypotension BP<90 or a drop in BP of 20 mm of Hg or more
- Tachycardia >140 bpm or Symptomatic bradycardia <40 bpm
- Neurological deterioration -Sudden drop in Glasgow Coma Scale (GCS) of more than 2 points
- Prolonged and/or frequent Seizures
- Drop in Urine output to less than 0.5ml/Kg of total body weight
- Any patient who you are worried about being a risk for acute clinical deterioration

Paediatric – Code Blue

Call a Paediatric Code Blue Medical Emergency if the following criteria are identified:

AIRWAY (ALL AGE)	Threatened
BREATHING AND CIRCULATION	Neonatal • <i>Respiratory Rate is less than 25 or more than 70</i> • <i>Pulse rate is less than 80 beats per minute or more than 170 beats per minute</i> • <i>Systolic BP less than 50 mm of Hg</i>
	INFANTS • <i>Respiratory rate less than 10</i> • <i>Pulse rate less than 60</i> • <i>Systolic BP less than 50 mm of Hg</i>
	Age 1-4 years • <i>Respiratory Rate less than 5</i> • <i>Pulse rate less than 60 beats per minute</i> • <i>Systolic BP is less than 55 mmHg</i>
	Age 5-11 years • <i>Respiratory Rate less than 5</i> • <i>Pulse rate less than 50 beats per minute</i>

	• Systolic BP less than 50 mm of Hg Age >12 years • Respiratory Rate less than 5 • Pulse rate less than 40 beats per minute • Systolic BP less than 70 mm of Hg For all age groups • Apnoea • Seizure • Bleeding • Sudden fall in Consciousness • Staff concerns about the patient • Low urine output (>0.5 mLs per kg bwt for 3 Successive Hours)
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NEONATAL AND OBSTETRIC – CODE BLUE

Call a Code Blue Neonatal and Obstetric if the following criteria are identified:

- Shoulder Dystocia
- Eclamptic Seizure
- Maternal cardiac arrest
- Significant Antepartum Hemorrhage (APH) or Post-Partum Hemorrhage (PPH)
- Prolonged fetal bradycardia
- Umbilical cord prolapses
- Maternal deterioration
- Staff or family concerns

Other medical emergency groups

In addition to the prescribed Code Blue events, additional emergencies have been designated as emergency events at AHS and will be activated via the “55” emergency phone number.

Code Blue – Neonatal

This code should be called when extensive neonatal resuscitation is anticipated or required, including, but not limited to, (unanticipated baby born in poor condition, external cardiac massage, prolonged IPPV management or support, infants requiring airway management).

The switchboard will page the following Code Blue team.

- Paediatric registrar
- Consultant Paediatrician

- Duty Anaesthetist
- Obstetric anaesthetist
- Paediatric RMO
- Shift coordinator Maud Bellas
- Clinical midwife labour ward
- Clinical midwife AAU
- NNU coordinator
- NUM paediatrics / neonatal
- Staff development nurse neonates
- Associate midwifery manager NAS
- Midwifery unit manager
- Hospital nurse manager
- MET/After hours CNS

An adult MET call may be required in addition to the above when the maternal condition may require additional support. Follow normal MET criteria as outlined in the code blue action cards

Code Blue – Obstetrics

This designated emergency is to be used for any Obstetric Emergency that poses a risk to the life or well-being of the mother and baby. This includes, but is not limited to:

- Antepartum haemorrhage
- Primary or secondary post-partum haemorrhage
- Eclampsia
- Shoulder dystocia
- Prolonged foetal bradycardia
- Perimortem caesarean section

Activation

1. Dial '55' and state 'Code Blue Obstetrics'
2. State your location, including room number

Paging distribution list

- Obstetrician
- Obstetrics GP
- Obstetric anaesthetist
- Duty anaesthetist
- Clinical Midwife - labour ward

- Shift Coordinator Maud Bellas' ward
- Clinical Midwife - Acute Assessment Unit (AAU)
- Associate Midwifery Unit Manager (MUM) Non-Ambulatory Services (NAS)
- Midwifery Unit Manager
- Hospital Nurse Manager (HNM)
- Paediatric Registrar
- Consultant Paediatrician
- MET/ After-hours CNS

Code Blue – Caesarean Section

This designated emergency is to be used when the obstetric consultant decides to proceed to a Category 1 Caesarean Section following assessment. The time and date of the decision must be documented in the patient's healthcare record.

Activation

1. Dial '55' and state 'Code Blue Caesarean Section'
2. State your location, including room number and 'Theatre 7'.

Paging distribution list

- Obstetrician
- Obstetrics GP
- Obstetric anaesthetist
- Duty Anaesthetist
- Paediatric Registrar
- Paediatric RMO
- Consultant Paediatrician
- Clinical Midwife AAU
- Associate MUM (NAS)
- Clinical Midwife - labour ward
- Shift Coordinator - Maud Bellas
- MUM
- Theatre team
- Nurse Unit Manager NUM - Theatre
- PCA morning support
- Maternity Patient Care Assistant (PCA)

- HNM
- NeoNatal Unit (NNU) Coordinator
- NUM paediatrics / neonatal
- After hours Clinical Nurse Specialist (CNS)/MET CNS (in hours)

Code Blue Response – Visitors, employees and non-admitted patients

Within the MET boundaries, a Code Blue can be called for a visitor, employee or non-admitted patient following the activation criteria.

If ongoing treatment is required, the visitor, employee or non-admitted patient will be transported to the AHS Emergency Department.

The Medical Emergency Record Form must be completed for all Code Blue medical emergencies involving visitors, employees, or non-admitted patients.

The MET Nursing / Midwifery Lead will ensure a copy of the form is provided to the CNS-Resuscitation and Safety Quality Unit via email:

AHS.METcoordinator@health.wa.gov.au.

Documentation Requirements

The following documentation requirements are mandated for all Code Blue medical emergencies:

- Senior Nurse on the MET is responsible for ensuring the Medical Emergency Record Form is completed
- The MET Team Leader will delegate a medical member to document the incident in the patient's healthcare record
- The original form is placed within the patient healthcare record, and a scanned copy is forwarded to the CNS-Resuscitation via the AHS.METcoordinator@health.wa.gov.au email address, as well as the Nurse / Midwifery Unit Manager for the area.

Operational Debrief

The relevant nurse/midwifery unit manager schedules and facilitates an operational debrief, including all employees involved in the Code Blue, as required. The AKG Deteriorating Patient and Resuscitation Committee provides governance over MET processes, including monitoring MET activations and identifying recommendations (as appropriate) to improve the MET response and management of the deteriorating patient.

ACTION CARDS

Code Blue Medical Emergency		PERSON IDENTIFYING THE CODE BLUE	Action Card
1.	Activate a Code Blue Medical Emergency. Dial 55 – State: • Code Blue Medical Emergency, • Type: Adult, Obstetric, Pediatric, or Neonate, • Location (ward/area) and room.		
Until the MET response arrives:			
2.	Commence Basic Life Support (BLS) if required within your capabilities.		
3.	Delegate a staff member to bring the resuscitation trolley to the scene.		
4.	Delegate a staff member to direct the MET members to the location of the emergency event.		

Code Blue Medical Emergency		Switchboard Operator	Action Card
1.	Receive call to raise Code Blue via the red phone receiver. Respond to all calls on extension '55' IMMEDIATELY – this takes priority over any prior or incoming calls.		
2.	Confirm the exact location of the Code Blue incident from the caller and determine if the location falls within the MET boundary.		
3.	Confirm the type of Code Blue requested as per 'Adult', 'Neonate', 'Paediatric', 'Obstetric' or 'Category 1 – Caesarean section'.		
4.	If the Code Blue has been activated inside of the MET service area, activate the appropriate MET team (via the specific paging group).		
5.	If the Code Blue has been activated outside of MET service area contact St John Ambulance (via 000) to dispatch an Ambulance to attend the incident.		
6.	Await and action any further instructions from the MET Team Leader in relation to the medical emergency which may include: Stand down the Code Blue Contact St John Ambulance to give an update of the patient's condition Other assistance as required.		
11.	Attend operational debrief as directed.		

Note:

Notification of medical emergency via an external call, i.e. (08) 9391 2000

- Respond as per actions 1–5.

Notification is sent directly to the reception counter.

- If someone presents to the main or emergency reception and states that they have collapsed in an external area on campus, such as the front car park, tell the person to wait at that reception and Page MET to attend reception.
- The person can then take the team to the person requiring medical assistance

Code Blue Medical Emergency		Nurse Allocated to Patient for the Shift PRIMARY NURSE	Action Card
1.	Attend to the patient immediately when informed of a Code Blue call.		
2.	Continue patient assessment and immediately commence CPR if required.		
3.	Handover to MET on arrival using iSoBAR format.		
4.	Assist and liaise with the team leader performing the A-E assessment		
5.	Ensure the Shift Coordinator is aware of the patient's condition.		
6.	Ensure the patient's notes and records are at the scene.		
7.	Remain with the patient during the Code Blue event and assist in treatment/resuscitation as required.		
8.	Liaise with the Shift Coordinator to ensure ongoing care for other allocated patients.		
9.	Ensure that an effective clinical management plan has been documented.		
10.	Attend operational debrief as directed.		

Code Blue Medical Emergency	SECOND RESPONDER	Action Plan
Activate a Code Blue Medical Emergency. Dial 55 – State Code Blue Medical Emergency, Adult, Obstetric, Caesar, Paediatric or Neonate, location (ward/area) and room.		
Until the MET response arrives:		
Ensure the MET Call is activated.		
Proceed to the designated area with MET Trolley.		
If the victim is unresponsive, assist the primary Nurse in doing CPR, and if not in arrest, assist the team leader in completing the Succinct A-E assessment.		

Code Blue Medical Emergency		Nursing Shift Coordinator	Action Card
1.	Ensure a Code Blue Medical Emergency has been called.		
2.	Ensure the following equipment is available at the event: • Resuscitation trolley • ECG machine • Glucometer. Arrange portable O ₂ and Suction as required.		
3.	Direct staff to adopt roles i.e., Airway, Compressions, Defibrillation, Drugs, Scribe, etc		
4.	Ensure the patient's healthcare records are available to the MET.		
5.	Ensure MET Leader is informed of the patient's Goals of Patient Care and/or 'Advanced Health Directives' that are in place.		
6.	In liaison with Medical Liaison, ensure that the patient's admitting team has been notified of the event.		
7.	Delegate / provide ongoing care for other patients and relatives if present.		
8.	Ensure the Nurse Unit Manager for the area is notified of the emergency.		
9.	If the admitting team is not available, ensure the patient's next of kin are informed of the event in liaison with the general medicine registrar (medical liaison).		
10.	Delegate cleaning and restocking of the resuscitation trolley and bed space.		
11.	Attend operational debrief as directed.		

Code Blue Medical Emergency Team Leader - Medical ICU Registrar Paediatric Consultant / Registrar / Obstetric Consultant Action Card	
1.	On notification of a Code Blue – attend scene immediately.
2.	On arrival at scene – identify self and designation.
3.	Identify self as Team Leader.
4.	Assess patient.
5.	Assist in treatment / resuscitation of patient as required.
6.	Provide critical care support, including management of the airway and intravenous access if required
7.	Liaise with the Medical Liaison for treatment and plan.
8.	Ensure a clinical management plan has been documented in collaboration with the admitting team, including arrangements for transfer to ICU, or another critical care facility where necessary.
9.	Liaise with MET Senior Nurse to confirm patient transfer arrangements, where required.
10.	Complete MET call documentation as required and sign.
11.	After review of the patient / situation the Team leader is the only person authorised to cancel a medical emergency call.
12.	Attend operational debrief as directed.

Code Blue Medical Emergency		General Medicine Registrar Medical Liaison	Action Card
1.	On notification of a Code Blue – attend scene immediately.		
2.	On arrival at scene – identify self and designation.		
3.	Identify Team Leader.		
4.	Assess patient.		
5.	Assist in treatment / resuscitation of patient as required. Ensure IV access and blood results are available / obtained.		
6.	Liaise with Duty Anaesthetist (Team leader) for treatment and plan.		
7.	Obtain / review patient's current admission record and notes (including Goals of Patient Care).		
8.	Liaise with the patient's admitting team and request attendance.		
9.	Ensure a clinical management plan has been documented in collaboration with the admitting team, including arrangements for transfer to ICU, or another critical care facility where necessary.		
10.	Liaise with MET Senior Nurse to confirm patient transfer arrangements, where required.		
11.	In liaison with the Shift Coordinator, if the admitting team is not available, ensure patient's next of kin are informed of event.		
12.	Attend operational debrief as directed.		

Code Blue Medical Emergency		Anaesthetic Registrar Airway Lead	Action Card
1.	On notification of a Code Blue, attend the scene immediately.		
2.	On arrival at the scene, identify yourself and your designation.		
3.	Identify Team Leader.		
4.	Assess the patient.		
5.	Assist in the treatment/resuscitation of the patient as required. Assist with medical liaison and Team leader in the diagnosis of the cause of deterioration.		
6.	Liaise with the MET Senior Nurse regarding plans for various advanced procedures.		
7.	Obtain/review the patient's current admission record and notes.		
8.	Ensure a clinical management plan has been documented in collaboration with the admitting team, including arrangements for transfer to the ICU or another critical care facility.		
9.	Can act as the team leader if the ICU registrar is unavailable to lead the team.		
10.	Attend operational debrief as directed.		

Code Blue Medical Emergency		MET Senior Nurse MET CNS (In-Hours) NUM Paediatric ward (Paediatric Code, In-Hours) AHCNS (Out of hours)	Action Card
1.	On notification of a Code Blue, attend the scene immediately.		
2.	On arrival at the scene – identify self and designation.		
3.	Assess the patient.		
4.	Identify Team Leader and ensure roles are being filled i.e. Team Leader, Airway, Compressions, Defibrillation, Drugs, Scribe, etc.		
5.	Ensure the Medical Emergency Record Form is in progress, and that the patient's notes and records are available.		
6.	Ensure all MET members are present; call 55 again if needed. Control the area and reassign non-essential staff.		
7.	Assist in the treatment/resuscitation of the patient as required.		
8.	Support staff are involved. Look for opportunities to train and develop staff.		
9.	Assist with patient transfer as required.		
10.	Ensure a clinical management plan has been documented in collaboration with the admitting team, including arrangements for transfer to the ICU or another critical care facility where necessary.		
11.	For transfer – assign/obtain ALS competent escort, O ₂ , suction and defibrillator.		
12.	When MET is completed, ensure a copy of the Medical Emergency Record Form is forwarded to the Nurse Unit Manager of the area. The original form is placed within the patient healthcare record and a scanned copy is forwarded to the CNS-Resuscitation via the AHS.METcoordinator@health.wa.gov.au		
13.	Attend operational debrief as directed.		

Code Blue Medical Emergency		Treating Team Registrar	Action Card
1.	Attend patient immediately when informed of Code Blue call.		
2.	Assist in treatment / resuscitation of patient in liaison with MET.		
3.	Ensure patient's family / relatives are informed of critical event.		
4.	Ensure a clinical management plan has been documented in collaboration with the MET.		
5.	Inform treating or on-call Consultant of patient's MET call.		
6.	Attend operational debrief as directed.		

Code Blue Medical Emergency		MET Scribe	Action Card
1.	Document all patient assessments, interventions and all drugs administered contemporaneously on the MET Record form		
2.	Ensure the General Medicine Registrar completes the AKMR3.1 before leaving		
3.	Ensure the Duty Anaesthetist and MET Senior Nurse sign the AKMR 3.1		
4.	Print name and designation and sign the AKMR3.1.		
5.	Ensure the resuscitation trolley is not left unattended until restocked and tamper-evident seal in place.		
6.	Attend operational debrief as directed.		

Code Blue Medical Emergency		MET PCA	Action Card
1.	On notification of a Code Blue, attend the scene immediately.		
2.	Inform the MET Senior Nurse / Shift Coordinator of your arrival.		
3.	Assist with equipment retrieval as required.		
4.	Deliver / collect equipment's, blood products as required and directed by the MET team		
5.	Assist with transfer of patient as required.		
6.	Only leave area once discussed with MET Senior Nurse.		
7.	Attend operational debrief as directed.		